RECURRENT
TEMPLATE FOR PRIOR AUTH/PRE-DETERMINATIONS
FOR USE BY PROVIDER THROUGH REQUEST TO LANTHEUS LINK

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[Date]

ATTN: [Dept Name]
[Name of Insurance Company]
FAX #: [fax #]

RE: Prior Authorization/Pre-Determination request for PSMA-Targeted PET Imaging with PYLARIFY®(piflufolastat F 18) Injection

ICD10 Diagnosis Code(s):

CPT / HCPCS Code(s):

Patient Name:
Subscriber ID #:
Group #:
Date of Birth (DOB):
Pending Ref #:

Dear Medical Reviewer:

I am writing to request prior authorization/pre-determination for Prostate-Specific Membrane Antigen (PSMA) Targeted PET imaging with PYLARIFY® (piflufolastat F 18) injection for my patient [patient name] who has suspected recurrent prostate cancer based on an elevated serum PSA level.

(Please include patient details that support authorization in the paragraph below (e.g. date of prostate cancer diagnosis, prior treatment history, PSA response to prior treatment, current PSA) and then complete this section with the provided language)

Patient has been diagnosed with prostate cancer on (date)…

- Prior treatments included (i.e., prostatectomy, radiation, ADT, etc) on date(s)
- PSA response (nadir) to prior treatment included…
- He now has suspected recurrence of prostate cancer with an elevated PSA of x…

(Refer back to payer policies for specific list of medical criteria to customize above)

Based on this assessment and current clinical guidelines, this patient has evidence of recurrent prostate cancer.

PSMA targeted PET imaging will assist in determining the location and the extent of his recurrent disease. The purpose of using PSMA-targeted PET imaging with piflufolastat F 18 injection is to detect and locate metastatic spread of cancer as PYLARIFY binds to the prostate specific membrane antigen protein found on the surface of over 90% of prostate cancer cells. Based on the results of PSMA-targeted PET imaging with PYLARIFY, we believe we will be able to recommend a more personalized and appropriate management plan for (name of patient).

Per review of your medical policy (enter #/title), (enter insurance co/RBM name) provides positive coverage for other radiopharmaceuticals, and we feel the same consideration should be given for this newly FDA approved radiopharmaceutical agent.

NCCN Clinical Practice Guidelines in Oncology – Prostate Cancer Version 4.2024 – May 17, 2024 indicates:

- F-18 piflufolastat (PYLARIFY®) can be considered as an alternative to standard imaging of bone and soft tissue for initial staging, the detection of biochemically recurrent disease, and as a workup for progression.
- Because of the increased sensitivity and specificity of PSMA-PET tracers for detecting micrometastatic disease compared to conventional imaging (eg, CT, bone scan) at both initial staging and biochemical recurrence (BCR), the Panel does not feel that conventional imaging is a necessary prerequisite to PSMA-PET and that PSMA-PET/CT or PSMA-PET MRI can serve as an equally effective, if not more effective front line imaging tool for these patients[1]

Approved by the FDA as of May 26, 2021, the labeled indication is as follows:

Piflufolastat F 18 injection is a radioactive diagnostic agent indicated for positron emission tomography (PET) of prostate-specific membrane antigen (PSMA) positive lesions in men with prostate cancer:

- with suspected metastasis who are candidates for initial definitive therapy.
- with suspected recurrence based on elevated serum prostate-specific antigen (PSA) level.

The CONDOR clinical study[2] was a Phase 3 prospective multicenter study of PYLARIFY (piflufolastat F 18) injection in 208 men with biochemically recurrent prostate cancer and uninformative standard imaging. The median baseline PSA was 0.8 ng/mL (range: 0.2-98.4 ng/mL), all patients underwent PSMA-targeted PET imaging scan with PYLARIFY (piflufolastat F 18) injection. The disease detection rate range was 59% to 66% (≥ 1 lesion detected per patient by piflufolastat F 18 PET/CT by 3 central readers). The correct disease localization rate (CLR) was 84.8%-87.0% (lower bound of 95% CI: 77.8%-80.4%).

In the CONDOR study, although the impact on patient management was not prospectively evaluated, 64% of subjects had a change in intended management of the disease, with 79% of the changes coming after a positive PSMA-targeted PET imaging with piflufolastat F 18 injection scan while 21% of the changes followed a negative piflufolastat F 18 injection scan highlighting the utility and actionability of the scan on treatment recommendations.[3]

Of the evaluable patients, 131 patients 44.3% (58 patients) had a change from salvage local therapy to systemic therapy, and 32.8% (43 patients) had a change from noncurative systemic therapy to salvage local therapy.[4]

The suboptimal diagnostic performance of conventional imaging in patients with prostate cancer often leads to ineffective under-treatment or unnecessary over-treatment. The selection of appropriate therapies is contingent upon the accurate assessment of the patient's "true" burden of disease.

Current conventional imaging has low diagnostic performance for detecting and locating early recurrent disease. It is estimated that 30-40% of patients will fail initial therapy and require additional treatment with many of these patients harboring occult metastatic disease[5,6]. Therefore, more accurate imaging at the time of diagnosis would help better

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[1] NCCN Clinical Practice Guidelines in Oncology – Prostate Cancer Version 4.2024 – May 17, 2024. Available at: https://www.nccn.org/login?ReturnURL=https://www.nccn.org/professionals/physician_gls/pdf/prostate.pdf

[2] Morris MJ, Rowe SP, Gorin MA, Saperstein L, Pouliot F, Josephson D, Wong JYC, Pantel AR, Cho SY, Gage KL, Piert M, Iagaru A, Pollard JH, Wong V, Jensen J, Lin T, Stambler N, Carroll PR, Siegel BA; CONDOR Study Group. Diagnostic Performance of ^{18}F-DCFPyL-PET/CT in Men with Biochemically Recurrent Prostate Cancer: Results from the CONDOR Phase III, Multicenter Study. Clin Cancer Res. 2021 Jul 1;27(13):3674-3682. doi: 10.1158/1078-0432.CCR-20-4573. Epub 2021 Feb 23. PMID: 33622706.

[3] Morris, et. al.

[4] Morris, et. al.

[5] Medical News Today. How nerves fuel prostate cancer growth. Available at: https://www.medicalnewstoday.com/articles/319789

[6] Paller CJ et al. *Hematol Oncol Clin North Am.* 2013;27(6):doi:10.1016/j.hoc.2013.08.005

identify patients likely to benefit from first-line potentially curative treatment while identifying patients with metastatic spread of disease who would no longer be candidates for curative intent therapies. PSMA-targeted PET imaging

improves disease assessment, which reduces of the risk of undertreatment (leading to disease spread) and overtreatment (leading to unnecessary toxicity or surgery).

Given PSMA-targeted PET imaging with piflufolastat F 18 injection's demonstrated ability to accurately detect prostate cancer lesions, both at initial diagnosis and at recurrence of the disease after treatment failure, utilization of piflufolastat F 18 has been shown to enable and improve disease assessment for patients for patients across the spectrum of Prostate cancer.

For the reasons stated above, I strongly believe that PSMA-targeted PET imaging with piflufolastat F 18 injection is appropriate and required for [patient name] and should be a covered benefit.

Thank you for your time and consideration. If you have any questions and/or concerns, regarding the procedure or this patient's medical condition, please do not hesitate to contact me at the number listed below.

We look forward to [payer name]'s timely, positive response.

Sincerely,

[MD Name]
[Practice Group Name]
[Street Address]
[City/Town, State Zip]
Phone #:
Fax #:
TIN:
NPI #:

Imaging/Hospital Facility info:
[Facility Name]
[Street Address]
[City/Town, State Zip]
Phone #:
Fax #:
TIN:
NPI #:

Enclosures

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